

1. Administrative & Study Identification * **Full Study Title:** A PHASE 1, OPEN-LABEL, DOSE ESCALATION AND DOSE EXPANSION STUDY TO EVALUATE THE SAFETY, TOLERABILITY, PHARMACOKINETICS, AND ANTITUMOR ACTIVITY OF PF-08052667 AS A SINGLE AGENT AND IN COMBINATION THERAPY IN PARTICIPANTS 18 YEARS OF AGE AND OLDER WITH BLADDER CANCER * **Short Title/Acronym:** A Study to Learn About the Study Medicine PF-08052667 in People With Bladder Cancer * **Protocol Number:** C6001001 * **NCT Number:** NCT07206225 * **Posting ID:** 160738293110869604 * **Sponsor Name:** Pfizer * **Investigational Product:** PF-08052667 (Integrin Beta-6 Directed ADC), Sasanlimab, and Bacillus Calmette-Guérin (BCG) vaccine * **Phase of Development:** Phase 1 * **Trial Status:** Recruiting * **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale * **2.1 Study Objective** * **Primary Objective:** To evaluate the safety, tolerability, pharmacokinetics, and antitumor activity of PF-08052667 as a single agent and in combination therapy with BCG and/or sasanlimab. * **Secondary Objectives:** To determine the recommended dose, evaluate Event-Free Survival (EFS), understand the body's processing of PF-08052667 (pharmacokinetics), and assess Objective Response Rate (ORR) and Overall Survival (OS). * **2.2 Background & Rationale** * To address the significant medical need in patients with high-risk non-muscle invasive bladder cancer (NMIBC) who have not responded adequately to standard Bacillus Calmette-Guérin (BCG) therapy or are ineligible/refuse radical cystectomy. The study explores direct intravesical administration of PF-08052667 to work directly on cancer cells while minimizing systemic side effects, potentially offering a bladder-preserving treatment.

3. Study Design & Methodology * **3.1 Design Framework** * **Study Type:** Interventional * **Control Type:** Single-arm (Dose escalation and dose expansion across 3 parts: Part 1 monotherapy dose escalation, Part 2 combination dose escalation, Part 3 dose optimization and expansion) * **Blinding:** Open-label * **Randomization:** N/A * **3.2 Planned Enrollment & Duration** * **Target N\$:** Approximately 294 participants * **Number of Sites:** Global (Multicenter, e.g., Emory University Hospital/Winship Cancer Institute) * **Estimated Study Start:** Mid-2024 / Early 2025 * **Estimated Primary Completion:** Up to approximately 2 years of active treatment plus long-term follow-up

4. Subject Selection (Eligibility Criteria) * **4.1 Inclusion Criteria** 1. 18 years of age or older (or the minimum age of consent per local regulations). 2. Histological diagnosis of high-risk, non-muscle invasive urothelial carcinoma of the bladder defined according to the WHO grading system as carcinoma in situ (CIS), with or without concurrent T1/Ta papillary disease. (Note: High-grade T1/Ta papillary disease, in the

absence of CIS, may be eligible for certain cohorts in Part 2 and 3). 3. BCG unresponsive and BCG-exposed cohorts should have persistent or recurrent disease after receiving at least 5 out of 6 doses of the BCG induction therapy. 4. Have refused or are ineligible or not appropriate for radical cystectomy. 5. ECOG Performance Status of 0 or 1, and available tumor tissue within the last 6 months. On-treatment tumor biopsy is optional unless mandated. * **4.2 Exclusion Criteria** 1. Concomitant anti-cancer therapy for Non-Muscle Invasive Bladder Cancer (NMIBC) and prior radiation therapy to the bladder. 2. Renal or hepatic impairment, and hematologic abnormalities as defined in the protocol. 3. Active, uncontrolled infection as specified in the protocol.

5. Intervention & Treatment Plan * 5.1 Investigational Regimen *

Dosage/Frequency: Administered on Days 1, 8, and 15 of each 21-day treatment cycle. Part 1 evaluates PF-08052667 monotherapy dose escalation. Parts 2 & 3 evaluate PF-08052667 in combination with BCG and/or sasanlimab. * **Route of Administration:**

PF-08052667 and BCG via intravesical instillation (injected directly into the bladder via a catheter); Sasanlimab via subcutaneous injection. * **Duration of Treatment:** Until disease progression, unacceptable toxicity, withdrawal, or up to about 2 years. * **5.2 Concomitant & Prohibited Therapy * Permitted:** Standard supportive care. * **Prohibited:** Concomitant anti-cancer therapy for NMIBC; prior radiation therapy to the bladder.

6. Study Timeline & Visit Schedule | Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Tissue Requirement (tumor tissue ≤ 6 months), Medical History, Baseline Labs | | **Treatment Cycles** | 21-Day Cycles | Clinic visits on Days 1, 8, 15 for intravesical instillation of PF-08052667, safety monitoring, optional/mandated on-treatment tumor biopsies, and PK tracking | | **End of Study / Follow-up** | Post-Treatment | Long-term follow-up after treatment discontinuation until loss to follow-up, death, or study termination |

7. Outcome Measures (Endpoints) * 7.1 Primary Endpoints * Number of participants with dose limiting toxicities (DLTs) in dose escalation (Part 1 and 2 participants only). * Number of participants with adverse events (AEs) characterized by type, frequency, severity (CTCAE v5.0), seriousness, and relatedness to study drug(s) (Part 1 and 2 participants only). * Number of participants with laboratory abnormalities characterized by type, frequency, severity (Part 1 and 2 participants only). * **7.2 Secondary & Exploratory Endpoints** * Pharmacokinetics: Maximum Observed Serum Concentration (C_{max}). * Pharmacokinetics: Time to Reach Maximum Observed Serum Concentration (T_{max}). * Pharmacokinetics: Minimum observed serum concentration (C_{trough}). * Efficacy measures such as Event-Free Survival (EFS), Overall Survival (OS), and Objective Response Rate (ORR).

8. Safety & Adverse Event Reporting * Adverse Event (AE)

Monitoring: Intensive Phase 1 safety monitoring, capturing dose-

limiting toxicities (DLTs) to establish safety profiles of monotherapy and combination therapy. * **Serious Adverse Events (SAEs)**: Standard reporting timeline (typically 24 hours). * **Data Monitoring Committee (DMC)**: Internal sponsor review committees tracking safety events for dose escalation clearing.

9. Statistical Analysis Plan (SAP) Summary * Analysis

Populations: Safety Analysis Set (all patients receiving ≥ 1 dose of study drug), Efficacy/Evaluable Set. * **Sample Size Justification**: $N = 294$ calculated to adequately power dose escalation (Part 1 & 2) tracking for MTD and to evaluate initial efficacy signals in dose expansion (Part 3). * **Handling of Missing Data**: Standard survival analysis censoring applied for Event-Free Survival analysis for patients lost to follow-up.

10. Quality Assurance & Regulatory Compliance * GCP Compliance:

This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. * **Monitoring Plan**: High frequency of onsite monitoring to track safety profiles due to the Phase 1 dose-escalation design. * **Audit Trail**: eCRF locking and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance * Primary ID: C6001001 *

Registry Number: NCT07206225 * **Posting ID**: 160738293110869604 * **Sponsor/Lead Organization**: Pfizer * **Study Title**: A Study to Learn About the Study Medicine PF-08052667 in People With Bladder Cancer * **Official Regulatory Title**: A PHASE 1, OPEN-LABEL, DOSE ESCALATION AND DOSE EXPANSION STUDY TO EVALUATE THE SAFETY, TOLERABILITY, PHARMACOKINETICS, AND ANTITUMOR ACTIVITY OF PF-08052667 AS A SINGLE AGENT AND IN COMBINATION THERAPY IN PARTICIPANTS 18 YEARS OF AGE AND OLDER WITH BLADDER CANCER

12. Clinical Framework (Bladder Cancer Specific) * Primary

Condition: High-risk, Non-Muscle Invasive Bladder Cancer (NMIBC) / Non-Muscle Invasive Bladder Neoplasms * **Condition Ontology Codes**: MeSH: D000093284, SNOMED: 425231005 * **Diagnosis Threshold**: Histological diagnosis of carcinoma in situ (CIS), with or without T1/Ta papillary disease * **Medical Methodology**: Intravesical Instillation and Subcutaneous Immunotherapy * **Investigational Drug ATC Code**: L03AX03 (BCG vaccine) * **Optimization Target**: Event-Free Survival, Bladder preservation / Prevention of radical cystectomy

13. Study Procedures & Methodology * Management Pathway:

Intravesical administration of PF-08052667 +/- standard BCG immunotherapy and subcutaneous sasanlimab * **Education Protocol (HCP)**: Protocol-specific training on correct catheterization and intravesical instillation procedures * **Education Protocol (Patient)**: Counseling on intravesical therapy expectations, retention time, and post-instillation care * **Quality Audit Mechanism**: Pharmacokinetic tracking, safety review committees for dose escalation, and optional centralized pathology/biopsy reviews

14. Observation & Follow-Up Schedule * **Total Duration:** Up to 2 years of treatment + long term survival follow-up * **Onsite Visit Cadence:** Days 1, 8, and 15 of every 21-day cycle * **Remote Monitoring Frequency:** Routine safety follow-up calls * **Checklist Review Interval:** Safety assessments prior to each dose escalation tier decision

15. Sign-off & Approval | Role | Name | Signature | Date | | :---
| :--- | :--- | :--- | | **Principal Investigator** | [Name] | _____ |
[DD-MMM-YYYY] | | **Clinical Operations Lead** | [Name] | _____ | [DD-
MMM-YYYY] | | **Lead Statistician** | [Name] | _____ | [DD-MMM-YYYY]
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